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Paper 1 of 2 — MONITORING AND EVALUATION OF HEALTHCARE SERVICES COURSE CODE - NSG 511

Level 500 • First Semester • Handout

3080

MONITORING AND EVALUATION OF
HEALTHCARE SERVICES

COURSE CODE - NSG 511

SESSION: 2019/2020

CREDIT:

DEPARTMENT OF NURSING SCIENCE, FACULTY OF CLINICAL
SCIENCES.

COURSE DESCRIPTION

- This course is designed to build capacity of the learners to be able to apply the knowledge of data collection and transformation to inform decision making in the process of monitoring and tracking of services and programmes that are provided by nurses.
- The contents includes concept of monitoring and evaluation, use of appropriate tools for assessment and the process of developing a monitoring and evaluation plan.
- Concept of monitoring and evaluation
- Data collection methods in informed decision making
- Use of appropriate tools for assessment
- Process of developing a monitoring and evaluation plan
- Case studies and practical experiences of monitoring of services provided by nurses

COURSE OBJECTIVES

- To explain the concepts “monitoring” and “evaluation”.
- To discuss why monitoring and evaluation activities are carried out
- To elaborate on the steps involved in carrying out monitoring activities
- To describe the link between monitoring and evaluation
- To explain the barriers to program evaluation
- To guide students to use the structure – process – output model to describe M&E of a definite healthcare service

Overview of M&E

- Monitoring and “evaluation (M&E) is essential management tools to assess health programme and services.
- It helps to ensure that health activities are implemented as planned and to assess whether desired results are being achieved.
- Monitoring progress and evaluating outcomes of health care services

are important to improve the performance of those responsible for implementing the services.

- The process of M&E show whether a service or program is accomplishing its goals.

Overview of M&E

- Monitoring and evaluation can sometimes seem like an unaffordable luxury, an administrative burden, or an unwelcome instrument of external oversight.
- But if used well, M&E can become a powerful tool for social and political change.

MONITORING

- Monitoring is a continuing function that uses the systematic collection of data on specified indicators to inform management and the main stakeholders of an ongoing International Federation or national society operation of the extent of progress and achievement of results in the use of allocated funds.
- Monitoring of a program or an intervention involves the collection of routine data that measure progress toward achieving program objectives.
- It involves counting what we are doing, and it involves routinely looking at the quality of our services. It is used to track changes in program performance over time.
- It is an ongoing, continuous process

MONITORING

- “Monitoring and Planning” goes together, one of the reasons for monitoring a program/intervention/service is to ensure that programs are implemented as planned and prepare for plan.
- The plan should specify what needs to be done, who is going to do it, and when it is to be done.
- Inadequacy in planning will result in inadequacy in monitoring.
- Monitoring is sometimes referred to as process evaluation because it focuses on the implementation process and asks key questions:

MONITORING

- To what extent are planned activities actually realized?
- What programs are provided, to whom, when, how often, for how long, and in what context?
- How well has the program been implemented?
- How much does implementation vary from site to site?
- What is the quality of the programs provided?

Purpose of Monitoring

- It helps in setting “norms of performance
- It helps in measuring the level of performance
- It helps in comparing performance level with standards or norms
- It helps in identifying deviations and explain the reasons for the deviation for taking necessary corrective action

Monitoring Process

- Monitoring process is a “continuous cycle that involves constant feedback. There are four steps in monitoring process:
1. Collecting and analysing data: the first step in the monitoring process is measuring, recording, collecting and analysing data on actual implementation of the programme.
 2. Detecting deviations from plans: following data analysis, any deviations from the plan is communicated to the programme managers.
 3. Diagnosing causes for deviations: the program manager detects the causes of the deviation after analysis.
 4. Taking corrective action: corrective actions are plan and implemented to achieve the desired goal (s), then the cycle begins again.

Steps in conducting monitoring activities

- Proposed by Mwadime et al, 1999
- Review existing information related to the project
- Develop/revise goals and objectives for the programme
- Familiarize with or develop conceptual framework for the project

Steps in conducting monitoring activities

- Identify monitoring objectives
- Identify indicators
- Determine which categories of workers, supervisors or other will be responsible for the collection of each category of monitoring data
- Develop a timetable for frequency of monitoring
- Develop/strengthen a management information system

Steps in conducting monitoring activities

- Train staff in monitoring activities
- Develop monitoring instruments
- Conduct monitoring activities
- Analyze monitoring and interpret monitoring data
- Make recommendations
- Implement recommendations
- Identify new indicators based on the recommendations
- Modify the monitoring system if necessary

- Continue monitoring

Levels of Monitoring

- Managers at the top level: They have to develop health plans based on objectives, goals, devise strategy and allocate necessary resources
- Managers at the middle level: They are more concerned with whether they are getting desired output from the inputs that are being utilized
- Managers at the operational level: They have to supervise actual operations and to ensure that planned activities are being carried out as per schedule

Domains of Information Required in a Monitoring System

1. Inputs—Resources going into conducting and carrying out the project or program. These could include staff, finance, materials, and time.
2. Process—Set of activities in which program resources (human and financial) are used to achieve the results expected from the program (e.g., number of workshops or number of training sessions).
3. Outputs—Immediate results obtained by the program through the execution of activities (e.g., number of commodities distributed, number of staffs trained, number of people reached, or the number of people served).

Example

- A country director is interested in finding out if the post-abortion care provided in public clinics meets national standards of quality
- This is also monitoring because it requires “tracking something (quality of care).
- A country director wants to know how many sex workers have been reached by your program this year. This is monitoring because it is concerned with counting the number of something (sex workers reached).

EVALUATION

- Evaluation is the “systematic and objective assessment of an on-going or completed operation, programme or policy, its design, implementation and results.
- The aim is to determine the relevance and fulfilment of objectives, as well as efficiency, effectiveness, Impact (overall Goal) and sustainability.
- An evaluation should provide information that is credible and useful, enabling the incorporation of lessons into management decision-making

EVALUATION

- Evaluation is a systematic way of learning from experience and using the lessons learnt to improve current activities and promote better

planning by careful selection of alternatives for future action.

- It measures how well the program activities have met expected objectives and/or the extent to which changes in outcomes can be attributed to the program or intervention.
- Evaluation is also the use of social research methods to systematically investigate a program's effectiveness.
- It is a fundamental exercise to help decision-makers understand how and to what extent, a program is responsible for particular measured results.

EVALUATION

- The difference in the outcome of interest between having or not having the program or intervention is known as its "impact," and measuring this difference and is commonly referred to as "impact evaluation."
- Evaluations require a well-planned "study design, data collection at the start of a program (to provide a baseline) and again at the end, rather than at repeated intervals during program implementation and a control or comparison group in order to measure whether the changes in outcomes can be attributed to the program.

Questions that are addressed by evaluation

1. What outcomes are observed?
2. What do the outcomes mean?
3. Does the program make a difference?

Questions that are addressed by evaluation

- Here are some of the questions that could answer with a process evaluation when implementing an initiative to address a syphilis outbreak among men who have sex with men (MSM):
- How did Disease Intervention Specialists (DIS) and STD program management collaborate with community-based organizations (CBOs) or other partners to reach MSM who engage in high-risk behaviours?
- How many CBO "outreach workers received STD training?
- What activities did these partners implement to address the problem?
- When were these activities conducted?

Questions that are addressed by evaluation

- Where were these activities conducted?
- Was the target population reached?
- What were the problems encountered in reaching the target population?

Uses of Evaluation

1. To assess the changes in the target group (e.g., changes in risk

behaviour)

2. To assess the extent to which objectives have been met.
3. It is the process of determining the effectiveness of a program or a project.
4. To review the implementation of the program and/or services provided by health programs so as to identify problems and recommend necessary revisions of the programme.
5. To assess progress towards desired health status at national or state levels and identify reasons for the gap, if any

Uses of Evaluation

6. Evaluation contribute towards better health planning
7. To document results achieved by a project funded by donor agencies.
8. To improve health programs and the health infrastructure
9. Allocation of resources in current and future programme
10. To track the outcomes and impacts of programs or projects at the larger population level,

Types of Evaluation

1. Formative evaluation evaluates a program during development in order to make early improvements.
 - It helps to refine a program.
2. Summative evaluation: Provides information on program effectiveness and is conducted after the completion of the program design.
 - It help decide whether to continue, end, or expand a program.
3. Process/implementation evaluation determines whether program activities have been implemented as “intended.

Types of Evaluation

4. Outcome/effectiveness evaluation measures program effects in the target population by assessing the progress in the outcomes or outcome objectives that the program” is to achieve.
5. Impact evaluation assesses program effectiveness in achieving its ultimate goals. Its Focuses on long-term sustained changes as a result of program activities, both positive and negative and intended and”

Barriers to evaluation

- Lack of time, knowledge /skills and fear of change
- Poor project design, e.g. M & E was not built in, and so would mean additional workload, and a restricted budget
- Stakeholders may expect positive results and question the value of monitoring and/or evaluation if it yields negative findings
- Stakeholders may be concerned that M&E results might be negative

- M & E is sometimes seen as “police work” or a fault-finding exercise
- Stakeholders may argue that M&E resources, not just money, but staff time, would be better spent on expanding the programme

Link between monitoring and evaluation

- Monitoring activities focus on programme inputs
- Monitoring also focuses on project activities or processes
- Monitoring focuses on outputs
- Evaluation, on the other hand, focuses on outcomes and impact
- Outputs and outcomes can be monitored on an ongoing basis and evaluated at the end of the programme

Link between monitoring and evaluation

Differences between monitoring and evaluation

Monitoring Evaluation

Continuous Periodic; at important milestones e.g. mid-term, end of term.

Keeps track of activities and documents progress In-depth analysis; compares planned versus achieved (objectives versus out puts, outcomes and impact).

Focuses on inputs, activities and outputs, and implementation processes, for example Focuses on outputs in relation to inputs, results in relation to cost, processes used to achieve results; participatory. overall relevance; outcomes, impact and sustainability.

Answers what inputs and activities were implemented and results achieved. Answers why and how results were achieved; and why not. Contributes to building theories and models for change.

Differences between monitoring and evaluation

Monitoring Evaluation

Focuses on planned results Captures on planned and unplanned results

Alerts managers to problems and provides options for corrective actions. Provides managers with strategy and policy options

Self-assessment by programme managers supervisors, community stakeholders and donors. Internal and /or external analysis by programme managers, supervisors, community stakeholders, donors and or external evaluators.

Usually accepts design Usually free to challenge design

M & E OF A PROGRAMME TO MANAGE MALNUTRITION

- Objectives;
- To improve the management of severe malnutrition among children
- To reduce case fatality rates of children due to severe malnutrition
- Monitoring activities;
- Input:
- Availability of trained staff;
- Availability of trained mothers

- Availability of supplies needed to care for these children (milk, antibiotics, etc.).

M & E of a programme to manage malnutrition

- Processes: How feeding activities are carried out to ensure that children get all required feeds;
- Recording of left-over feeds;
- Re – offering feeds to children who do not finish their feeds during routine feeding time;
- Inserting a naso-gastric tube to feed children with poor appetite
- Giving prescribed antibiotics
- Feeding children 3-hourly day and night

M & E of a programme to manage malnutrition

- Output:
- Number of children who receive feeds 3 hourly day and night
- Outcome:
- Increase in the number of nurses who implemented the monitored activities
- Evaluation:
- Compare the % of children who had died at baseline to those dying after pediatric staff had received training on the implementation

Paper 2 of 2 — MONITORING AND EVALUATION OF HEALTHCARE SERVICES COURSE CODE - NSG 511

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THE ROLE OF PUBLIC HEALTH NURSES IN IMPROVING ADOLESCENT HEALTH

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ABSTRACT Adolescent Health (AH) is an integral part of public health with the goal of improving healthy development, safety and well-being of adolescents because the behavioural patterns established during this period influence the health status of the adolescents and the risk of developing chronic diseases during adulthood. Adolescence being a time of great physical and psychological change, many social and physical conditions as well as behavioral pattern can impact adolescent health and heighten the risk of illness and diseases. This makes adolescents to be susceptible to various health issues by nature because of their stages of development, which affects their ability to make safe decisions, and the need to rely on significant others for support. Several other factors such as desire for greater autonomy, pressure to conform to peers, exploration of sexual identity, and increased access to and use of technology, violence and sexual abuse, social and cultural norms, family circumstances, emotional and physical environment of the adolescents contribute significantly to health issues being faced by the adolescents. Therefore, nurses have significant roles to play to ensure optimal growth and development of the adolescents and to prevent and reduce the impact of changes associated with adolescence on the health and well being of the adolescents. This paper presents the concept of adolescence, components of adolescent health, health-related challenges associated with adolescence and role of public health nurses.

KEY WORDS Adolescence, Public Health, Nurses, Emerging

1 Introduction health [3]. During this phase, adolescents establish patterns of behaviour related to diet, physical activity, substance use, and sexual activity that can affect their health and the health of others around them and subsequently put their health at risk now because the behavioural patterns established during this period determine the health status of the adolescents [4-5]. However, despite being

thought of as a healthy stage of life—there is significant risk of developing chronic health conditions during adulthood [1]. Adolescence is a years that are preventable and treatable [6]. critical transitional phase of growth and development. In addition, this stage is characterized by transition between childhood and adulthood usually between the age of 10 and 24 years and characterized by physical and psychological development from these years including mental disorders, substance abuse, smoking, nutrition and weight conditions, sexually transmitted infection including human immunodeficiency virus (HIV), teen and unintended pregnancy, homelessness, academic problems and related challenges dropping out of school, homicide, suicide [8-10]. Adolescents are a vulnerable population and are though there are many effective policies and programs that address adolescent health related issues cause of their stages of development which affects such as access to health care, school-based health care services, prevention of substance use, violence, and delinquencies, mental health and substance use intervention as well as HIV prevention. Adolescence and delinquencies, mental health and substance use intervention as well as HIV prevention. Adolescence and delinquencies, mental health and substance use intervention as well as HIV prevention. Adolescence and delinquencies, mental health and substance use intervention as well as HIV prevention.

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munodeficiency virus (HIV), teen and unintended pregnancy, homelessness, academic problems and related challenges dropping out of school, homicide, suicide [8-10]. Adolescents are a vulnerable population and are though there are many effective policies and programs that address adolescent health related issues cause of their stages of development which affects such as access to health care, school-based health care services, prevention of substance use, violence, and delinquencies, mental health and substance use intervention as well as HIV prevention. Adolescence and delinquencies, mental health and substance use intervention as well as HIV prevention. Adolescence and delinquencies, mental health and substance use intervention as well as HIV prevention. Adolescence and delinquencies, mental health and substance use intervention as well as HIV prevention.

es[19-20]. Also[adolescents can be particularly vulnerable due to risk factors that contribute to stress. These suggest that the leading causes of illness during adolescence[such as a desire for greater autonomy]pressure to conform to peers[exploration]able or treatable. Nevertheless[to ensure adequate of sexual identity]and increased access to and use growth and development of adolescents in good of technology[violence and sexual abuse[21]. This health there is need for information including age suggests that the higher the numbers of risk factors appropriate comprehensive health and sexuality education[adolescents are exposed to]the greater the potential for health conditions and challenges[21]. services that are acceptable[equitable]appropriate. In view of adolescents vulnerability to health-related conditions[all nurses should have the knowledge to assess and intervene]even if they are not trained adolescents'healthcare professionals. Nurses are perfectly positioned to get to know the adolescent and their families[learn their history and family dynamics]and provide education and Furthermore[there are certain issues that put adolescent health on the priority list of public health which can influence how the health of the adolescents will be approached in the coming decades[14]. It is however important that the public vulnerable adolescents in all healthcare set-

healthcare providers especially public health nursing [16-17]. Engaging in formal and informal assessments have the understanding that the population of the adolescents that can identify adolescents at risk for adverse health conditions is becoming more ethnically diverse. However, to improve the health of the adolescents, nurses must first understand the complexity of the many risk factors contributing to adolescent health, the impact of early recognition of adolescent health condition, and the use of valid assessment and screening tools to identify adolescent vulnerability to diseases and illnesses and implementation of interventions to prevent complications [17].

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2 Components of Adolescent Health so that government agencies, organizations and individuals who work with youth can support the

Adolescence is a significant period for health and healthy development of all adolescents [10].

cent but it is often overlooked. According to Shee-

han et al. [23] in both low/middle and high countries about 42 million adolescents who are be-

Adolescence is considered a critical period of between the ages of 10 and 19 are generally healthy.

human development with associated rapid physi-

In the recent years, there has seen a reduction in cal psychosocial, emotional and cognitive devel-

sexual risk behaviors, teenage pregnancy and opment as well as sexual and reproductive matura-

births, smoking, and use of some substances, as well as higher academic achievement for younger adolescents. Though biology growth impacts adolescent health and development, however, social contexts including families, media, schools, and neighborhoods where adolescents live, learn, and grow also have a great influence on their health and well-being [24].

ing [12][9][14].

Adolescents' health focuses on the health care Social and cultural norms, Social and cultural needs of the adolescents, health conditions and norms, particularly regarding gender, as well as ethnicity, sexual orientation, or disability, significantly determine social patterns of behaviour and developmental complications associated with specific health problem and assist the adolescents and their families to come up with strategies to deal with chronic health issues both now and in future. of school, experience sexual violence and have restricted opportunities due to early pregnancies [29]. Also, attitudes associated with masculinities contribute to boys engaging in more risky behaviours, them to set long-term goals and make rational decisions, like harmful substances use, with greater exposure in the same vein their heightened neurobiological flexibility and resilience make them vulnerable to harmful or unhealthy influences that can set

Adolescent independence and autonomy [Dur- them on less positive paths [25]. Additionally [ac- ing adolescence [there is a significant increase in cording to United Nations International Children's Emergency Fund [UNICEF [26] adolescent health relations become more significant [9 [14]. In the includes changing transitions within multiple do- same vein [family and parents or caregivers have a mains such as physical [social [emotional [cogni- great influence on adolescent health. Nevertheless [tive [and intellectual. The fast-paced development family connectedness protects the adolescents from of these different domains can lead to phenomenal multiple health risks [31-33].

growth occurring at different rates [which can put Impact of family circumstances [Family situa- adolescents at a higher risk for risk-taking behav- tions can also impact the health of adolescents [iors and emerging mental health issues [27]. There- through violence and abuse [impairment of identity fore [it is important to understand adolescent devel- development such as attitudes towards gender and opment [environmental influences [the risk and sexual identity [harmful practices like child mar- protective factors that can affect adolescent health riage [female genital mutilation/cutting [or paren- [http:// www. nursrxiv. org. cn](http://www.nursrxiv.org.cn)

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tal absence and migration [34]. Families can also mild to moderate depression and 5. 7% definite de-

serve as a strong influence for accessing education [pression among urban dwelling adolescent students

health services [and other health resources. with higher incidence of depression been reported Social [emotional and physical environment in females and older adolescents [36-37]. Adolescent of adolescents [The social [emotional and physical mental health issue was suggested to be associated

environments in which adolescents live and learn with low socioeconomic class [parental separation [

have significant influence on their health. More being from a single parent or polygamous home [significantly is transition from climate change to previous experience of violence [low physical ac- armed conflict [migration and economic down- tivity/sedentary lifestyles [overweight/obesity and

ticks. Also adolescents are sometimes distinctive- previous history of sexual abuse [36-37].

ly affected by the risks and opportunities emerging Patton et al. [14] stated that around the world from urbanization and globalization [28]. health services and policies have largely ignored ad- Technological advancement New technology olescence. Estimates of 1. 2 million adolescents and increase access to social media provide oppor- die each year largely from preventable causes [12].

tunities for information and services but can also This age group has failed to experience the im- reinforce vulnerabilities including exposing ado- provements other segments of the population have

lescents to bullying sexual abuse depression or seen. For instance while under five deaths halved mental health conditions. during the Millennium Development Goals period

Habit formation Adolescents are also vulnera- progress in adolescent mortality stalled [32-33].

UNI-

ble to the commercial environment especially as it CEF [26] also reported that since 2010 AIDS-relat-

relates to food tobacco and alcohol advertisement ed deaths among children have halved yet have de-

and sales [26]

. More so factors related to physical creased by only five percent among adolescents.

environment such as road and playground safety Young women especially vulnerable to complica-

access to footpaths and parks water and sanitation tions of unsafe abortion and pregnancy are less

and pollutants also influence adolescents health and well covered by services than older women [38-39].

wellbeing [14]. Mortality and disability-adjusted life years

[DALYs] lost due to road injuries which is the

4 Adolescents' Health Issues

leading cause of deaths in adolescence are not de-

Long-term or chronic health problems during clining particularly among adolescents in Low/ adolescence period often make physical and emo- Middle Income Countries as they become more mo-

tional development more complicated. Illnesses bile [28]. The followings are the common health is-

injury medical treatment hospitalization and sur- sues associated with adolescence.

geries associated with chronic illnesses can cause Adolescent Stress

anxiety and emotional trauma to the adolescents. Most often adolescent period can seem like a

According to World Health Organization whirlwind of rapidly changing emotions. In fact

[WHO] diseases and injury experienced by ad-

[12]

some earlier theories about adolescent development

olescents have been found to be mostly avoidable proposed that "storm and stress" was to be expect-

yet neglected. In 2015 adolescents globally contributed and suggested adolescents characteristically contribute about 60% of the 135.6 million years lived tended to over-react to everyday situations^[40]. with disability and about half of global adolescent More so according to Shaikh^[40] and Deka^[42] it

mortality ^[45%] occurred in Africa^[35]. Evidence was stated that what may appear as "storm and suggested high prevalence of mental health issues stress" is actually the natural outcome of youth among in-school adolescents while 16.3% was re-learning to cope with a much larger array of new reported among rural in-school adolescents^[23.8%] and unfamiliar situations. However, the ability to <http://www.nursrxiv.org.cn>

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adaptively cope with stress is influenced by many factors. Depression among adolescents^[54]. Depres-

factors. Certain genetic factors such as temperament in adolescents is usually triggered by more fre-

ment make some people more sensitive to stress. Frequent exposure to social stressors peer victimiza-

On the other hand, certain environmental factors such as family and community can help to mitigate the effect of stress by enabling youth to become more resilient in the face of stress^[43]. This invariably contributes to a type of long-term neurological development that

. underlies emotion processing and regulation in so-

However, certain protective factors can reduce the effect of stress and increase adolescents' resilience during the stressful situation because resilience vulnerability to or worsening of depressive symp-

ment adolescents will experience fewer negative reactions among adolescents^[55]. actions negative behaviour in response to stress Additionally, anxiety is a significant behav-

and fewer adverse consequences of stress .

^[44-45]

behavioural problem among adolescents and many ado-

More so, social support provided by family, peers, teachers, adolescent health care providers

and young people met the criteria for anxiety disorders. According to Beesdo et al^[56] the will assist the adolescents in handling stressful and most frequent anxiety disorders among adolescents

challenging circumstances . This in turn will

^[46]

are separation anxiety disorder and specific and so-

give the adolescents sense of safety and security in social phobias occurring more frequently in females

copied with stress couple with favourable cultural than males. The heightened expectations on social

practices and environmental factors [45-50]

. acceptance in adolescents can trigger a correspond-

Behavioural health issues ing fear of rejection that contributes to social anxi-

Adolescent sometimes exhibit some behav- ety and depression. Evidence suggest that for some ioural and mental disorders such as suicide[depres- adolescents[physical symptoms of anxiety such as

sion[anxiety[substance use disorders and eating stomach or headaches may go unrecognized[but disorders. Evidence suggests that suicide is the with hormonal changes during this period symp- leading cause of death among people aged 10-34 toms may also manifest as behavioural defiance in-

years and suicide ideation intensifies between the stead of the more typical cognitive symptom such

ages of 12 and 17 years with almost two-thirds of as worrying[57]. Therefore[understanding adoles-

adolescents moving from ideation to planning and cents vulnerability to anxiety disorders is critical to

more than three-fourths from ideation to attempt better address the needs of adolescents already diag-

during the first year of onset of ideation [51]

. The nosed and to reduce future prevalence of the condi-

risk of suicidal behaviour can be attributed to neuro- tion.

biological mechanisms[however[suicidal behav- Furthermore[substance use and addiction iour can also be genetically inherited [52]

. Environ- pose a significant threat to adolescent health. This

ment and context factors also play a significant role behavioural problem is characterized as a problem-

in suicidal risk[especially the quality of familiar re- atic pattern in the use of an intoxicating substance

lationship and experience of the adolescents from that results in significant impairment and dis- the family members .

[53]

tress[58-59]. An important aspect of substance abuse

Depression is also a serious behavioural health in adolescence is comorbidity with other mental issue among adolescents and this is sometimes asso- health conditions such as anxiety and depres-

ciated with early life adversity. Early-life adversi- sion[60]. This is directly associated with neighbor-

ties contribute disproportionately to many health hood social disorganization such as level of crime[

problems[both physical and mental[and increase disinvestment and population turnover and social both metabolic dysregulation and treatment-resis- capital like the degree to which the member of the

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community are engaged in communal life are significantly related to opioid misuse in the adolescents. Nurses are part of healthcare work force that interacts [61]. However, Wilson et al. [62] stated that to ensure effective delivery of health care services and help adolescents overcome substance use addiction, health care providers must create a health care system. Nurses being at the centre of health care system and services play a significant role in providing effective care to the adolescents. This is very crucial to ensure confidentiality which is found to be central to facilitating honest disclosure from adolescents about all sensitive matters in- especially in regard to the unique healthcare need of the adolescents. Therefore to ensure optimal growth and development of adolescent nurses are expected to identify the these unique needs of the adolescents and ensure timely intervention. Eating disorders and Nurses serve as adolescents' educators, advocates, counselors, confidants and in matters relating to their health. It is also the responsibility of nurses to guide adolescents' parents to appreciate development and mental health as well as family that every child is unique and should prevent unhealthy peer and romantic relationships when adolescents are dissatisfied with their physical appearance, it can hamper their ability to complete the physical and socio-emotional tasks of development as well

cents on what is required to live a healthy lifestyle as damage their physical and mental health. Body including behavioural, sexual and reproductive dissatisfaction in some adolescents is associated formation. More so, it is the responsibility of nurses with changes and alteration in body image coupled to reach out to adolescents in the community, at with a greater tendency towards social comparison home, school and religion gathering to provide in- and stronger pressure to conform to gender intensive education on developmental processes of roles [63]. More so, obese and overweight adolescents, sexual and reproductive health as well cents are vulnerable to body dissatisfaction due to as societal expectation during this period. In view weight stigma, weight-based harassment and teasing of this, nurses must ensure that adequate informing, bullying and victimization especially for tion is given to the adolescents on personal hygiene girls [64]. This is also applicable to underweight including menstrual hygiene, adequate nutrition, boys, therefore, weight-based victimization experience and ill effect of illicit use of drugs. rience among peers increase adolescents' risk of Moreover, nurses are in the best position to low self-esteem and poor body image, bully and identify and manage adolescent health issues and teasing associated with weight gain also increase should be able to liaise between parents and their adolescent risk of mental and physical health out- adolescents because sometimes adolescent parents comes such as depression, anxiety, suicidality, are insensitive to the health care needs of their self-harm behaviours, substance abuse and disorders and most often they hoard information from dered eating behaviours [64]. Eating disorders such the adolescents especially in matter related to pu- as anorexia nervosa, bulimia nervosa, sub-threshold and sexuality. Therefore, nurses can guide old anorexia nervosa and sub-threshold bulimia the parent to identify problems being faced by the

nervosa and sub binge eating disorders often associated with adolescents and they should be guided to be close associated with body dissatisfaction and affect all genders, friends with them and facilitate good communication across races, ages and weight statuses

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tion. Lastly, nurses can as well liaise with the ment. " Adolescent Nutrition. Springer Cham schools to provide a safe and conducive physical 2020. 3-17.

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